

PsyClick Secure

User Manual

Clinician Manual & Patient Guide

Clinical Decision-Support System for Detecting Single-Session Psychomotor Disturbances
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Important safety and privacy notice

PsyClick is non-diagnostic decision support. It does not replace clinical judgment, risk assessment, or emergency procedures. Use only approved synthetic, de-identified, or consented data. Never treat a Green, Amber, or Red flag as a diagnosis.

How this manual is organized:

- **Part A — Clinician Manual:** for the clinician or researcher who operates PsyClick during a session.
- **Part B — Patient Guide:** plain-language information for the participant. A clinician may read it aloud or hand it over before a session.

Part A — Clinician Manual

Everything you need to run a session safely, from sign-in to sign-out.

A1. Before you begin

- Use a supported, up-to-date workstation with endpoint protection, automatic screen lock, and restricted physical access.
- Sign in with your own clinician account. Never share credentials, and never use a development or test bypass.
- The first account created on a new installation becomes the administrator; further accounts are created by an administrator.

A2. Start the application and sign in

1. Launch PsyClick and wait for the “API ready” indicator before signing in.
2. Enter your clinician ID and password. A successful sign-in opens the dashboard and records an audit event.

Good to know

Your password is stored using bcrypt hashing (minimum 12 characters), never in plain text. Sessions expire automatically when idle and can be revoked at any time. PsyClick runs entirely on this workstation — no internet connection is required.

A3. If sign-in is blocked (this is normal protection)

- After 5 failed attempts on one account, that account locks for 15 minutes.
- If many failed attempts come from one source within 5 minutes, the login is briefly throttled (about 2 minutes) and a security alert is written to the audit log.

This is expected brute-force protection, not a fault. Wait for the cooldown, confirm the password, and try again. Report repeated, unexplained lockouts to your administrator.

A4. Create a client and record consent

1. Create or select a pseudonymous client identifier. Avoid names, addresses, or other direct identifiers.
2. Explain the purpose, what is captured, how long it is kept, the limits, and the non-diagnostic nature — or hand the participant Part B of this manual.
3. Record the participant’s informed consent before any telemetry is captured.

The system enforces consent: capture will not start until consent is recorded (a fail-closed check on the server).

A5. Run the session

1. Complete keyboard calibration, then mouse calibration, following the on-screen prompts.
2. Administer PHQ-9 and GAD-7 according to your approved study or clinical protocol.
3. Run the emotional-task prompts. Supervise the participant throughout, and stop if they show distress or the system becomes unstable.
4. Let the session finalize. Do not close the application during analysis or while data is being saved.

A6. Read the dashboard and report

Review the T² score and threshold, PSI and PAI, confidence, the domain and difficulty-level profiles, and the plain-language rationale. The overall signal is shown as a colour:

Green	Amber	Red
Within baseline No unusual psychomotor pattern for this person this session.	Some deviation Interpret together with clinical context and observation.	Marked deviation Warrants closer clinical attention — never a diagnosis.

Always compare the output with your own clinical observation and record contextual factors. A flag is a signal to look more closely — never a diagnosis on its own.

A7. Export and handle reports

- Export only when necessary, and only to an approved, protected destination.
- Exports and backups are protected with Windows DPAPI encryption and recorded with a SHA-256 integrity check.
- Treat every report as restricted clinical and behavioural information. Never email or upload an unprotected report.

A8. Audit, log out, and close

- Use the Audit page to review login, consent, access, export, configuration, and backup events. The audit log is tamper-evident (hash-chained).
- Log out at the end of every session, and lock the workstation whenever it is unattended.
- Securely delete temporary files and follow your approved retention schedule.

A9. Administrator tasks

- Provision least-privilege accounts using the three roles: administrator, clinician, and auditor.
- Review failed logins, intrusion-monitor alerts, and unusual exports.
- Create and verify encrypted backups, and perform periodic restore drills.
- Activate the incident-response plan for any suspected data disclosure, malware, consent failure, data corruption, or unauthorized access.

A10. Troubleshooting

- **“PsyClick API is not available.”** Restart the application and confirm the backend is running on the loopback address **127.0.0.1:5101**. The app runs offline; no internet is needed.
- **“Too many failed attempts from this source.”** The intrusion monitor throttled that source after **8 failures in 5 minutes**. It clears automatically after about 2 minutes. Do not disable the monitor.
- **A session failed or was interrupted.** Preserve the audit information, do not reuse an uncertain partial report, and notify the administrator. Never expose database files or credentials while troubleshooting.

Deployment note

PsyClick is a research prototype. The security controls above are implemented and tested. Before real clinical deployment, complete the external evidence described in the CS0029 Security Document: an

independent installation test, an authorized live penetration test, a restore drill, and signed sign-offs.

Support and escalation

Report security incidents immediately to the Incident Coordinator, System Administrator, Data Protection Officer / Privacy Lead, and Clinical Lead, using the response plan in the CS0029 Security Document.

Part B — Patient Guide

A short, plain-language guide to what taking part involves. Please ask your clinician anything you're unsure about.

What is PsyClick?

PsyClick is a research tool that looks at how you type and move the mouse — the timing and rhythm of your movements, not the words you write. It helps a clinician notice small changes in focus or coordination. It is not a diagnosis, and it never decides anything about your care on its own.

Taking part is your choice

- Joining is completely voluntary, and you can ask questions at any time.
- You can pause, stop, or withdraw at any point, for any reason, without any penalty.
- Nothing is recorded until you give your consent.

What we collect — and what we don't

- **We collect:** The timing of your key presses and mouse movements, and your answers to two short, standard questionnaires (PHQ-9 and GAD-7).
- **We don't collect:** We do not keep the full text of what you type.
- **Your identity:** You are identified by a code, not your name.

What happens in a session (about 10–15 minutes)

1. A short keyboard and mouse warm-up (calibration).
2. Two brief questionnaires about how you have been feeling.
3. A few short typing tasks.

A clinician stays with you the whole time. Tell them if you feel uncomfortable — you can pause or stop whenever you like.

Your privacy

- Your information stays on the clinic's computer. It is not sent over the internet.
- Reports are encrypted and protected, and only authorized clinicians can open them.

What the results mean

At the end, the clinician sees a Green, Amber, or Red signal. This is only a prompt for them to look more closely — it is not a diagnosis and not a judgment about you. Your clinician always combines it with their own professional judgment.

Questions or concerns

Please ask your clinician, or contact the study's Clinical Lead or Privacy Lead. Remember, you may withdraw your consent at any time.